



Protocol Intelligence Report

Structured protocol review, comparable cohort benchmarking, recommendations, and audit-ready reporting.

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Prepared for clinical development, trial design, and operational planning review.

Classification
Internal review draft
File: trial_protocol_report.pdf

1. Executive Summary

- Protocol title: Prospective observational study of the treatment of mechanical low back pain using High-Intensity Focused Ultrasound (HIFU) of the lumbar medial branch.
- Clinical focus: Mechanical low back pain
- Study structure: Observational | Not provided
- Profile status: Reviewed with medium extraction confidence
- Comparator evidence: 502 matched studies | 285 completed | 22 disrupted.
- Decision posture: Mixed precedent posture (completed fit 35.8%, disrupted fit 38.2%).
- High priority: Finalize the missing core design fields before using this draft for governance review or external discussion.
- Medium priority: Rework the endpoint package or strengthen the narrative for why a weaker precedent pattern is appropriate.

2. Senior Decision Signals

Decision Area	Signal	Implication	Evidence
Evidence base	Strong	Confidence is highest when completed and disrupted comparators are both adequately represented.	502 total studies 285 completed 22 disrupted.
Overall posture	Mixed precedent posture	This is the clearest top-line read on whether the draft is behaving more like successful or disrupted precedent.	Completed fit 35.8% Disrupted fit 38.2%.
Enrollment plan	Within completed-trial range	Enrollment targets outside the completed range need stronger feasibility justification.	Target 75 Completed IQR 34.0 to 84.0.

Decision Area	Signal	Implication	Evidence
Execution exposure	Lower observed disruption exposure	Higher disruption prevalence should trigger more explicit mitigation planning.	Disrupted-status share 4.4%.
Protocol completeness	Specification gaps remain	Missing design fields make the benchmark less trustworthy and reduce report defensibility.	Phase, Allocation
Lead action	High	Finalize the missing core design fields before using this draft for governance review or external discussion.	Missing or unspecified fields: Phase, Allocation.

3. Executive Action Register

Priority	Category	Action	Recommendation	Evidence
High	Protocol completeness	Clarify	Finalize the missing core design fields before using this draft for governance review or external discussion.	Missing or unspecified fields: Phase, Allocation.
Medium	Endpoint Focus	Challenge	Rework the endpoint package or strengthen the narrative for why a weaker precedent pattern is appropriate.	Completed match 62.1% versus disrupted match 72.7%.

4. Reviewed Protocol Profile

Field	Value
Title	Prospective observational study of the treatment of mechanical low back pain using High-Intensity Focused Ultrasound (HIFU) of the lumbar medial branch.
Condition	Mechanical low back pain
Sponsor	Fundació Monclínic
Study Type	Observational
Phase	Not provided
Planned Enrollment	75
Arms Count	Not provided
Allocation	Not provided
Masking	Single
Intervention Model	Single Group Assignment

Primary Purpose	Treatment
Comparator	Immediate clinical response and possible adverse effects are evaluated
Target Population	Patients with mechanical low back pain
Geography Focus	Not provided
Start Date	Not provided
Completion Date	Not provided
Endpoint Focus	Not provided
Primary Endpoints	Spanish version of the Oswestry Disability Index at 1, 3 and 6 months.
Secondary Endpoints	Pain scores, Oswestry scale, catastrophizing scale, Brief Pain Inventory.

5. Comparative Narrative

Matched 502 studies for Mechanical low back pain, including 285 completed comparators and 22 disrupted comparators.

Top-line posture: Mixed precedent posture (completed fit 35.8%; disrupted fit 38.2%).

Enrollment position: Within completed-trial range. Protocol target 75 | completed-trial IQR 34.0 to 84.0 | disrupted-trial IQR 0.0 to 30.8.

Most important design differential: Endpoint Focus shows 62.1% completed-trial match versus 72.7% disrupted-trial match.

Lead action: Finalize the missing core design fields before using this draft for governance review or external discussion.

Interpretation note: this output is intended to support senior design review, not replace clinical, statistical, operational, or regulatory judgment.

6. Comparator Cohort Definition

Metric	Value
Clinical condition	Mechanical low back pain
Matched trial count	502
Completed comparators	285
Disrupted comparators	22
Active comparators	113
Evidence strength	Strong
Risk-status share	4.4%

Metric	Value
Median site count	1.0
Median country count	1.0

7. Success vs Disruption Benchmark

Domain	Protocol Draft	Completed Precedent	Disrupted Precedent	Signal
Enrollment	75	Median 53.0 IQR 34.0 to 84.0	Median 11.0 IQR 0.0 to 30.8	Within completed-trial range
Planned Duration	Not provided	Median 9.1 mo IQR 4.4 to 18.3 mo	Median 31.4 mo IQR 16.0 to 38.6 mo	Insufficient evidence
Phase	Not provided	Not available	Not available	Protocol field not specified
Study Type	Observational	13.3%	18.2%	Weak precedent signal
Allocation	Not provided	Not available	Not available	Protocol field not specified
Masking	Single	30.5%	22.7%	Mixed precedent
Intervention Model	Single Group Assignment	0.0%	0.0%	Weak precedent signal
Primary Purpose	Treatment	73.3%	77.3%	Mixed precedent
Endpoint Focus	Patient Reported	62.1%	72.7%	Mixed precedent

8. Decision Scorecard

Metric	Value
Matched cohort size	502
Completed comparator count	285
Disrupted comparator count	22
Evidence strength	Strong
Completed precedent fit (%)	35.8
Disrupted precedent fit (%)	38.2
Net precedent gap (%)	-2.4
Overall posture	Mixed precedent posture
Enrollment target	75
Completed enrollment IQR	34.0 to 84.0
Disrupted enrollment IQR	0.0 to 30.8
Completed median duration (months)	9.1

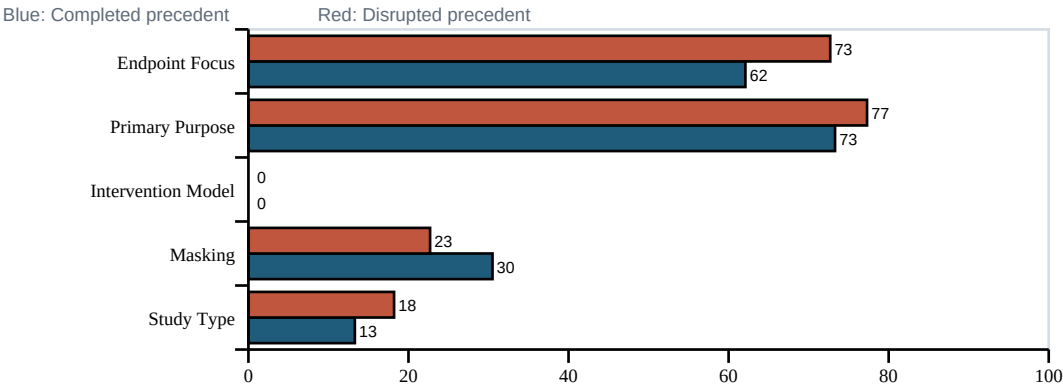
Metric	Value
Disrupted median duration (months)	31.4
Median site count	1.0

9. Design Differential Matrix

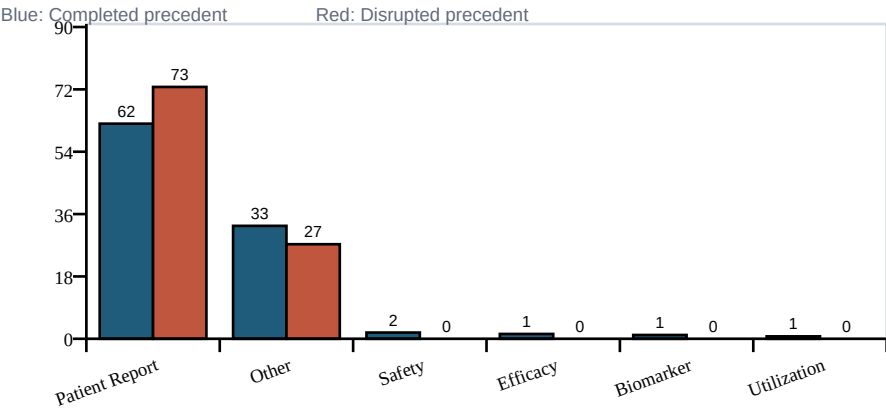
Domain	Protocol Choice	Completed %	Disrupted %	Gap %	Signal
Phase	Not provided	Not available	Not available	Not available	Protocol field not specified
Study Type	Observational	13.3	18.2	-4.9	Weak precedent signal
Allocation	Not provided	Not available	Not available	Not available	Protocol field not specified
Masking	Single	30.5	22.7	7.8	Mixed precedent
Intervention Model	Single Group Assignment	0.0	0.0	0.0	Weak precedent signal
Primary Purpose	Treatment	73.3	77.3	-4.0	Mixed precedent
Endpoint Focus	Patient Reported	62.1	72.7	-10.6	Mixed precedent

10. Key Figures

Completed vs Disrupted Precedent by Design Domain



Endpoint Focus in Completed vs Disrupted Comparators



11. Recommendation Detail

Priority	Category	Recommendation	Rationale	Evidence
High	Protocol completeness	Finalize the missing core design fields before using this draft for governance review or external discussion.	Key design fields are still unspecified, which weakens benchmark interpretation and report defensibility.	Missing or unspecified fields: Phase, Allocation.
Medium	Endpoint Focus	Rework the endpoint package or strengthen the narrative for why a weaker precedent pattern is appropriate.	Endpoint focus determines what decision the trial can credibly support.	Completed match 62.1% versus disrupted match 72.7%.

12. Comparator Exemplars

Comparator Lens	Status	NCT ID	Title	Enrollment	Sponsor
Completed Precedent	Completed	NCT04609254	Paracetamol, Dexketoprofen and Ibuprofen In Non-Traumatic Acute Low Back Pain	210.0	Pamukkale University
Completed Precedent	Completed	NCT04491175	Impact of DuoTherm on Opioid Use and Low Back Pain	160.0	MMJ Labs LLC
Completed Precedent	Completed	NCT04110119	Effectiveness of Chiropractic Application for Acute Low Back Pain	98.0	Medipol University
Completed Precedent	Completed	NCT02312895	Comparing Dry Needling to Manual Therapy for Patients With Mechanical Low Back Pain	73.0	Youngstown State University
Disrupted Precedent	Terminated	NCT01085448	Clinical Prediction Rule for Clinical Lumbar Instability	20.0	Drexel University
Disrupted Precedent	Terminated	NCT04990518	Physical Deconditioning in COVID-19 Positive Patients and Non-Specific Low Back Pain	350.0	University Hospital, Geneva
Disrupted Precedent	Terminated	NCT03600532	Teachable Moment Brief Intervention for Suicide Attempt Survivors in an Inpatient Setting	7.0	University of Tulsa
Disrupted Precedent	Terminated	NCT03634527	Auricular Point Acupressure: Examining the Scientific Underpinnings of Pain Relief	15.0	Johns Hopkins University
Active Context	Not Yet Recruiting	NCT07306364	Psilocybin-Assisted Physical Therapy in Chronic Low Back Pain	45.0	Yale University
Active Context	Enrolling By Invitation	NCT06892977	Mapping Stress and Pain Interactions (G072323N)	140.0	University Ghent

14. Audit Trail and Provenance

2026-03-15 11:53 UTC Extract Protocol Profile
Actor: System Artifact: Document ID: Prot_SAP_000.pdf
Extracted protocol profile from Prot_SAP_000.pdf.
2026-03-15 11:53 UTC Review Protocol Profile
Actor: User Artifact: Protocol Profile
Reviewer updated and confirmed protocol profile fields.
2026-03-15 11:53 UTC Build Comparable Cohort
Actor: System Artifact: Comparison Cohort
Built registry comparison cohort for Mechanical low back pain.
2026-03-15 11:53 UTC Export Pdf Report
Actor: System Artifact: Report ID: trial_protocol_report.pdf
Generated trial_protocol_report.pdf.

15. Methodology Notes

Source trials were retrieved from ClinicalTrials.gov and normalized into a comparator cohort. This report separates completed comparators from disrupted comparators and uses those lenses to assess how the reviewed draft aligns with stronger or weaker precedent patterns. Protocol fields were derived from uploaded documents using deterministic heuristics and, when configured, an LLM-assisted structured extraction step. All outputs are intended to support expert review rather than replace clinical, statistical, operational, or regulatory judgment.